



Fiona Stanley Fremantle Hospitals Group

OFFICIAL

Guideline

Management of consumers intending to operate a motor vehicle whilst intoxicated

Reference #: FSFH-MENH-GUI-0013

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital	Addiction Prevention & Treatment Service	Medical, Nursing, and Allied Health

1. Introduction

Road traffic accidents are the ninth leading cause of death across all age groups globally. Driving under the influence accounts for 27% of all road injuries. In addition, road traffic crashes constitute an important public health problem with significant individual and societal health and socioeconomic costs¹.

This document provides clinicians working in the FSFHG Addiction Prevention and Treatment Service (APTS) a structured approach to managing issues of risk when intoxicated drivers present for review.

2. Terminology

Intoxicated driver	Person who operates a vehicle after the intake of alcohol or drugs, causing disturbances in conscious level, cognition, perception, judgement, affect, behaviour or other psychophysiological functions and responses
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3. Guideline

Intoxicated drivers pose an immediate threat to themselves and to public safety. Health providers have a duty to exercise reasonable care to ensure appropriate steps are taken to protect the safety of the individual and the community. Refer to [Appendix 1](#) for a flowchart illustrating the process for managing intoxicated drivers who present to APTS.

- A healthcare practitioner/clinician should assess any consumer who presents to the service and appears to be intoxicated.
- Assessments may include:
 - observation of clinical/physical signs of substance intoxication (see [Appendix 2](#))
 - breath alcohol concentration
 - urine drug screen (UDS)
 - history from consumer or family member
 - intention to operate a motor vehicle
- If the staff member is unsure/unclear of the assessment, they should consult with their Clinical Coordinator/Manager.
- Refer to history from the consumer or family member to ascertain whether the consumer is intending to operate a motor vehicle
- If following assessment, the clinician deems that the consumer is intoxicated and intends to operate a motor vehicle:
 - the patient should be advised not to operate the motor vehicle.
 - alternative travel options should be outlined (e.g., offer to contact NOK to collect the consumer, arrange taxi services).
- If the consumer is deemed to be unwell and not safe for travel, he/she should be referred to the Emergency Department.
- If the consumer refuses all abovementioned options and there is an immediate or imminent risk, inform them that WA Police (WAPOL) will be notified as duty of care to ensure consumer and community safety.
 - Provide WA Police with the consumers name, date of birth, location of departure including the date and time, location of arrival (if known) and vehicle registration number (if known).
- All discussions should be documented including conversations with all parties involved including the consumer/family, police, senior clinician, or clinical coordinator.

3.1. Legal Considerations

- There is no mandatory requirement for health practitioners or staff to report or notify authorities of a driver under the influence of alcohol or other substances.
- To protect confidentiality, it is an offence under the Health Services Act (HSA) 2016 to collect, use or disclose any information obtained by a person because of their role [HSA section 219(1)(a)]².
 - However, a person does not commit an offence under the HSA in certain circumstances if the collection, use or disclosure meets certain requirements [HSA section 219(3)]².
- In relation to driving under the influence of alcohol or another substance, to be authorised under the HSA the collection, use or disclosure of information must be in **good faith** [HSA, section 220(1)] and only if reasonably necessary to lessen or prevent a:
 - serious risk to the life, health or safety of any individual [r 5(1)(a) of the Health Services (Information) Regulations]., (Regulations) and / or
 - real or immediate risk of danger to the public [r 5(1)(b) of the Regulations]¹.
- In these circumstances, WA Police are the appropriate authority to contact.
- There is **no mandatory obligation** for clinicians or medical practitioners to notify the authorities about an intoxicated driver³.
- It is the **responsibility of a driver** to report medical conditions that may impact driving tasks to the Department of Transport³.
- If there is a consumer who is deemed to be unfit to drive and poses an immediate threat to themselves or others, they may be referred to the Department of Transport as long as this information is provided in good faith and an opinion is formed as a result of having carried out a test or examination.
- In the event a patient has been advised not to drive due to a medical condition, and is known to continue doing so, the Department of Transport should be notified in formal written correspondence. This is typically drafted by the SMHS Legal Services Department.

3.2. Practical considerations

- Where a consumer has been assessed as intoxicated and unfit to drive by the relevant clinicians, they will be advised to abstain from driving.
- All discussion and correspondence must be clearly documented in the consumer's medical record.

- Staff should NOT offer to transport a consumer to their home.

4. Compliance/performance monitoring

The APTS Head of Service will be responsible for monitoring compliance with this guideline. Compliance will be monitored via routine clinical incident review processes.

5. Related policy documents

Department of Health:

- [Alcohol and Other Drug Withdrawal Management Policy MP0062/17](#)

FSFHG:

- Alcohol and Other Drugs: management of addiction, toxicity and withdrawal (FSFH-HW-POL-0063)

6. Related standards

- NSQHS Standards
 - Clinical Governance

7. Related legislation

- The Health Services Act 2016 (WA) (HSA)
- The Health Services (Information) Regulations 2017 (WA) (Regulations)
- Road Traffic (Administration Act) 2008 (WA)

8. References

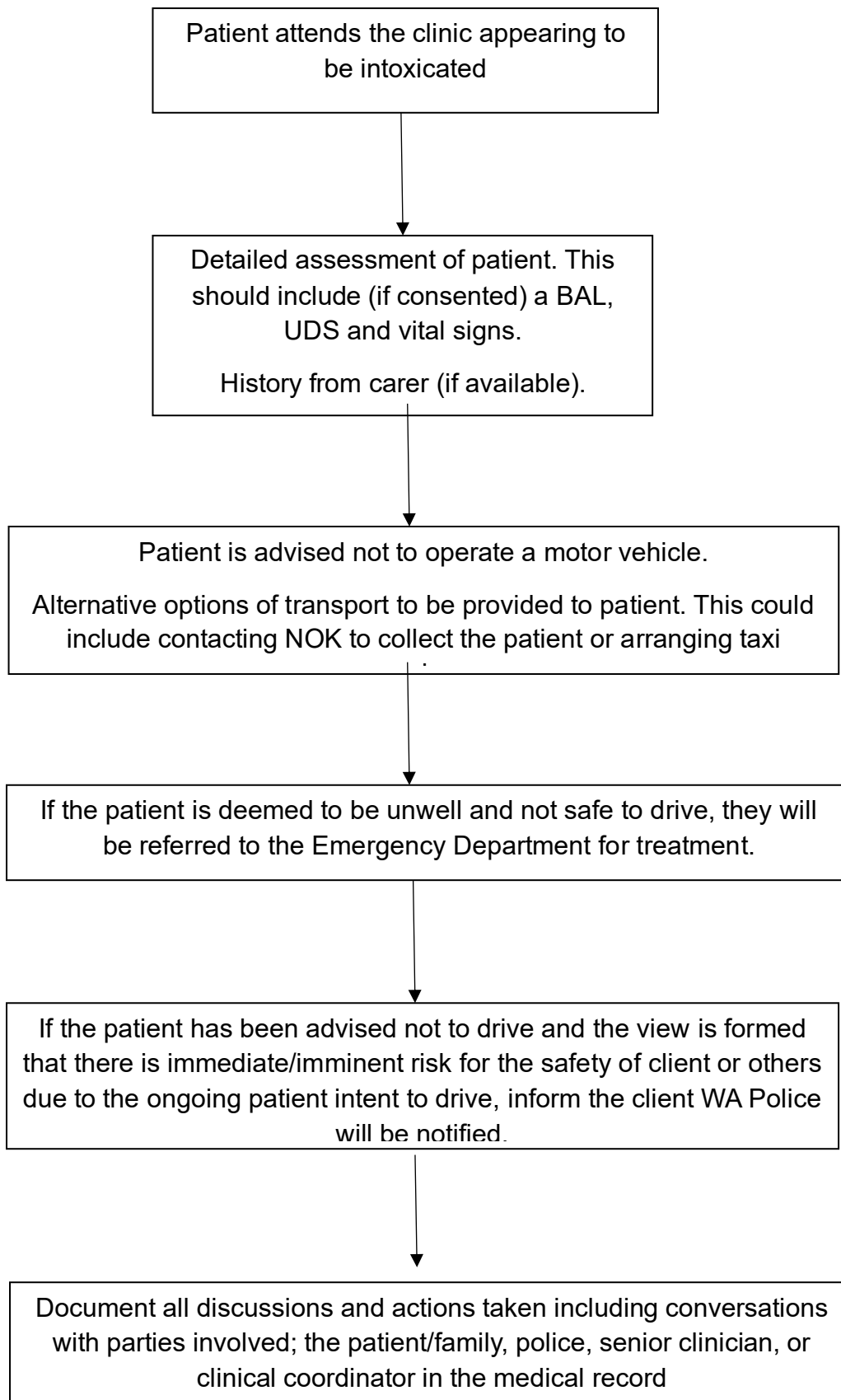
1. Basili E, Caschili C, Chiara BD, Pellicelli M 2024. The impact of road accidents on hospital admissions and the potential of ADAS in containing health expenditure: Evidence from Piedmont data. Transportation Research Interdisciplinary Perspectives 25 101125.
2. Government of Western Australia 2017. Health Services Act 2016. Version 00-e0-06
3. National Transport Commission 2022. Assessing fitness to drive for commercial and private vehicle drivers. Medical standards and clinical management guidelines. Austroads Ltd.

9. Authorisation

EXECUTIVE SPONSOR: Nurse Director Service 2					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1.0	July 2025	Consultant Psychiatrist	Service 5 SQR Committee	Service 5 SQR Committee	Aug 2028

10. Appendices

10.1. Appendix 1: Management of intoxicated drivers presenting to ATPS



10.2. Appendix 2: Common signs of intoxication

- Rambled or slurred speech
- Altered perception.
- Poor concentration or confusion
- Excessive restlessness or agitation
- Disorientation
- Unstable mood
- Paranoia
- Ataxia – repeated stumbling or an inability to walk in a straight line
- Euphoria – elation, talkativeness, and impulsiveness